

## Complaints Policy

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|------------------------------|-----------------------------------------|
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| <b>Policy Lead(s):</b>       | Judith Morris                           |
| <b>Designation:</b>          | Head of Patient and Carer Experience    |
| <b>Responsible Director:</b> | Chief Nursing Officer                   |
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## **1. Introduction**

This complaints handling policy describes how the core expectations given in the NHS Complaint Standards will be put into practice.

This policy sets out how we handle complaints and the standards we will follow. This process follows the relevant requirements as given in the Local Authority, Social Services and National Health Service Complaint Regulations 2009 and the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014 (the 2009 and 2014 Regulations).

It should be read in conjunction with the more detailed guidance modules available on the Parliamentary and Health Service Ombudsman website.

## **2. Purpose**

The purpose of the Complaints Policy is to provide guidance for raising, dealing with, and managing concerns and complaints; ensuring that they are fully and appropriately investigated and responded to. In addition, the Trust will ensure that, through this Policy, all staff understand their own responsibilities in relation to supporting those who raise a concern or complaint and its subsequent management. As a public document this policy also ensures that patients, carers, and the wider public understand how to raise a concern with the Trust. The purpose of the Complaints Policy is not to address staff complaints; members of staff may raise their concerns, complaints, and grievances via the:

- Freedom to Speak Up Policy
- Grievance Policy
- Dignity at Work Policy

## **3. Definitions**

### **3.1 Complaint**

A complaint or concern is defined as: 'Any expression of dissatisfaction, about a Trust service or member of staff, who is undertaking duties on behalf of the Trust.'

### **3.2 Complainant**

For the purpose of this policy, people raising concerns or complaints as patients, carers or their representatives will be referred to as a complainant.

### **3.3 Local Resolution**

This is the investigation and resolution of a concern or complaint made informally, to the service, where staff are able to resolve any issues locally without the need to escalate outside of the clinical area.

### **3.4 Patient Advice and Liaison Service (PALS)**

The Patient Advice and Liaison Service manages informal complaints and concerns and offers advice and support to complainants regarding the process of raising a concern and making a formal complaint.

### **3.5 Working Day**

In relation to the length of an investigation, the expression of a working day refers to days that exclude weekends and bank holidays.

### **3.6 Allocating Officers**

An individual identified as the lead person within a directorate responsible for identifying Investigating Officers (IO).

## **4. Duties / Responsibilities**

### **4.1 Chief Executive Officer**

The Chief Executive Officer is ultimately accountable for the quality of care within the Organisation, and therefore, as part of governance arrangements, needs an overview of all recorded dissatisfaction being expressed by patients and carers. The outcome of all formal complaint investigations will be submitted to the Chief Executive Officer, who will consider the investigation and sign off the final response to the complainant. When the Chief Executive Officer is unavailable to sign the letter, it will be countersigned by a nominated person acting on the Chief Executive Officer's behalf.

### **4.2 Chief Nursing Officer and Deputy Chief Executive Officer**

The Chief Nursing Officer has been designated by the Trust Board to take overall responsibility for the governance arrangements relating to this policy and will ensure that reports and issues are escalated to the Trust Board and its subcommittees as appropriate.

### **4.3 Trust Board**

The Trust Board is required to receive assurance that robust systems are in place that enables feedback to be heard, actioned and lessons learned, in order to provide the best possible care leading to an improved patient experience / service.

### **4.4 Safety and Quality Committee**

The Safety and Quality Committee has responsibility for ensuring that complaint handling throughout the Trust is monitored on behalf of, and provides assurance to, the Trust Board. The committee has responsibility for receiving and approving monthly, bi-monthly, and annual reports on complaints, PALS, and compliments.

### **4.5 The Patient and Carer Experience Group (PCEG)**

The PCEG will support the Trust Board in assuring that complaints are investigated, reviewed, acted upon appropriately and that lessons learned are implemented and monitored. The group supports the development and implementation of PALS and Complaints

procedures/policies and action plans and will act as a point of contact and support for Complaint Investigator, Directorate Leads and wider Trust staff.

#### **4.6 Director of Nursing**

The Director of Nursing is responsible for the strategic oversight of PALS and Complaints through the Head of Patient and Carer Experience. The Director of Nursing is responsible for presenting complaints reports at the Safety and Quality Committee, the strategic development of the PALS and Complaints service and action plans, policy development, and liaising with Associate Directors on strategic and complex issues.

#### **4.7 Head of Patient and Carer Experience**

The Head of Patient and Carer Experience is responsible for the leadership of the Patient Experience Team who deal with the day to day handling of complaints and concerns, and for liaising with operational staff across the Trust.

The Head of Patient and Carer Experience will be the first point of contact for any external Organisation's enquiries regarding complaints and is responsible for liaising with the Ombudsman (PHSO) and Care Quality Commission (CQC) in relation to complaints that cannot be resolved locally to the complainant's satisfaction. If the Trust receives notification that a case has been referred to the Ombudsman, the Head of Patient and Carer Experience will be the Trust's point of contact for the PHSO and responsible for managing responses to the PHSO.

The Head of Patient and Carer Experience will prepare the Annual Complaints Report, and other reports, on complaints for the Safety and Quality Committee, the Trust Board and the Care Quality Commission.

The role and responsibilities of the Head of Patient & Carer Experience and relevant delegated functions when dealing with complaints are set out complaint toolkit and appendix A in this policy.

#### **4.8 The Patient Experience Team**

The Patient Experience Team will provide advice and support to staff when dealing with informal concerns and formal complaints. The team will offer support to staff and complainants. All contact details are available on the Complaints Toolkit and the Trust website. The role and responsibilities when dealing with complaints are set out in the complaint toolkit and appendix A in this policy.

#### **4.9 Director of Operations**

The Director of Operations have overall responsibility for the implementation of this policy within their directorate. This involves ensuring that all staff are fully conversant and compliant with the policy.

The role and responsibilities of the Director of Operations when dealing with complaints are set out in the complaint toolkit and appendix A in this policy.

**4.10 Associate Directors of Nursing**

The Associate Directors of Nursing role and responsibilities when dealing with complaints are set out in the complaint toolkit and appendix A in this policy.

**4.11 Investigating Officers (IO)**

The role of the Investigating Officer is to undertake an investigation into the issues raised by the complainant. They are responsible for the initial and ongoing liaison with the complainant in the complainant's preferred manner, ensuring a thorough, fair, factual and timely investigation is carried out into the complaint.

The role of an Investigating Officer is detailed further in the Complaints toolkit and appendix A in this policy.

**4.12 All Staff including Temporary Staff**

All staff, including temporary staff, must comply with this policy and should treat all concerns and complaints seriously and address them as a matter of priority. Staff must ensure they take time to listen and ensure that they fully understand the concerns, asking for clarification where things are unclear. Staff must reassure the complainant that complaints are welcome as a means of enabling the service to improve, respond to the issue raised and provide an answer to the issues quickly or refer the complaint to someone who can assist them further. Staff must ensure that the concern or complaint is recorded and passed onto the PALS and Complaints Team via the feedback form located in the Complaints Toolkit.

## **5 Process**

**5.1** Staff speak to people who use our services every day. This can often raise issues that our staff can help with immediately. The Trust encourages patients, and their families and carers to discuss any issues they have with staff, as staff may be able to sort the issue out to the patient's satisfaction quickly and without the need for them to make a complaint.

Not all complainants want to raise a formal complaint. It is in the interests of patients, carers and healthcare professionals that patient and carers concerns and complaints arising from their care and treatment are resolved as quickly, efficiently and professionally as possible.

## **5.2 Complaints Process**

The complaints process will provide staff with support and knowledge to resolve concerns raised by complainants. Prompt action to resolve concerns and complaints can prevent them escalating into more serious cases. The process will ensure that the complainant is given the opportunity to understand all possible options for pursuing the complaint and what these options mean.

## **5.3 How Complaints and Concerns can be Raised**

Patients, carers, and their representatives can make a complaint or raise a concern to any member of staff –

- Verbally
- In writing
- Electronically by email

The Trust will consider all accessibility and reasonable adjustment requirements of people who wish to make a complaint in an alternative way and will record any reasonable adjustments made.

## **5.4 Timescales for Making Complaints**

Complaints must be made within 12 months of the date the incident being complained about happened or the date the person raising the complaint found out about it, whichever is the later date.

If a complaint is made to after that 12 month deadline, it can be considered if:

- there were good reasons for not making the complaint before the deadline, and it is still possible to properly consider the complaint.

Where timescales are not extended, complainants will be advised of their right to refer the complaint to the Parliamentary and Health Service Ombudsman (PHSO) for a review of this decision.

## **5.5 Who may Complain or Raise a Concern**

Concerns or complaints can be raised by existing or former patients or from a person acting on their behalf, or any persons who are affected by or likely to be affected by the action, omission, or decision of the Trust.

The Trust is committed to promoting equality and diversity. No Patient, or any other person involved in the investigation, or resolution of a concern or complaint, will receive unfair treatment as in line with the Trust's Equality and Diversity Policy. Reasonable adjustments should be offered to any person who may be at a disadvantage for any of these reasons. Information must be provided in a format and/or language that the individual understands (e.g. braille, easy read or sign language). Staff should be aware that it can be particularly

difficult for patients to take forward complaints due to mental ill health or fear that this may impact on the quality of care and support they receive. A patient may request the support of an independent advocate to raise a complaint on their behalf. The Patient Experience team will work with advocates and third parties to ensure complainants are supported.

If the person affected does not wish to deal with the complaint themselves, they can appoint a representative to raise the complaint on their behalf. There is no restriction on who may represent the person affected. However, they will need to provide us with their consent for the representative to raise and discuss the complaint with us and to see their personal information.

If the person affected has died, is a child or is otherwise unable to complain because of physical or mental incapacity, then the complaint may be made on their behalf by a representative. There is no restriction on who may act as representative but there may be restrictions on the type of information that can be shared with them.

In all circumstances where a representative is making a complaint, the complaint can be considered by the Trust if the representative is a relative or the other person has had or has sufficient interest in the welfare and the complaint is made in the best interests of the person on whose behalf the complaint is made. In the circumstances where the Trust does not intend to consider a complaint, the complainant will be notified of the reasons for this decision in writing.

#### **5.6 Complaints Raised by Members of Parliament (MP) on Behalf of Constituents**

Where a complaint is made on behalf of a patient by an MP it is assumed that implied consent has been given by that patient. If, however, the complaint relates to a third party, consent will need to be obtained from the patient prior to the release of personal information. These are acted upon in the same way as a letter of formal complaint, other than we aim to reply to the MP within 20 working days.

#### **5.7 Complaints Received from Care Quality Commission (CQC) on Behalf of Patients Detained under a Section of the Mental Health Act**

Where a complaint has been made by the Care Quality Commission on behalf of a patient detained under the Mental Health Act or subject to a Community Treatment Order, it will be assumed that implied consent has been given by that patient. If, however, the complaint relates to a third party, consent will need to be obtained from the patient prior to the release of personal information. These are acted upon in the same way as a letter of formal complaint. The response letter will be sent to the complainant and a copy forwarded to the CQC.

## **5.8 Complaints and Concerns Stages**

### **5.8.1 (Stage 1) - Local Resolution by Service**

The aim of local resolution is to manage concerns and resolve them at source. Staff should always attempt to deal with complaints swiftly, at the informal stage so that the complaint is resolved more quickly for the complainant.

Concerns raised should be listened to by any member of staff and, wherever possible, the member of staff to whom these were expressed should aim to provide an acceptable answer or explanation or refer the matter onto someone who can. Staff should complete the Complaints feedback form available in the Complaint Toolkit and forward to the PALS Team.

Stage 1 – local resolution concerns should be resolved within 3 working days or within timescales agreed by the complainant.

### **5.8.2 (Stage 2) Patient Advice and Liaison Service (PALS) - Supported Resolution**

The Patient Experience Team provides a supported and facilitated approach to resolving concerns and will act independently when handling patient concerns. The team will provide advice and assistance to staff experiencing difficulties in addressing any specific complainant concerns and/or problems that require an informal resolution.

On receipt, The Patient Experience Team will assess the complaint to identify any issues that may be of a serious nature and require further support or immediate input from a clinical team. The Patient Experience Team will advise the complainant on the PALS process and provide information on expected outcomes and timescales as outlined in the PALS and Complaints Toolkit.

Stage 2 – PALS concerns should be resolved within 5 working days or within timescales agreed by the complainant.

### **5.8.3 (Stage 3) Formal Complaints Process**

Where it has not been possible to resolve the issue, or where it has not been deemed appropriate through initial assessment/triage, or where the complainant requests a formal complaint, the Trust will then proceed to the formal stage of the complaints process. Formal complaints will be required to be completed and responded to within 35 working days. Please refer to the complaint's timeline in the complaints tool kit.

### **5.8.4 (Stage 4) Review of Formal Complaints**

The Trust will consider all requests to review formal complaint investigations and findings where a complainant is dissatisfied

with the response.

The Patient Experience Team will seek to outline the best approach to conduct the review. This will usually involve allocating a Reviewing Officer to follow the formal complaints process and work with the initial Investigating Officer to discuss the findings and review the outcomes. A further letter will then be written and signed by the Chief Executive Officer (CEO) detailing the finding of the review.

#### **5.9 Formal Complaint Acknowledgement**

A formal complaint will be acknowledged in writing or via email within 3 working days. The acknowledgement will contain details of the Trust formal process and information on other Organisation's that may be able to offer support whilst the investigation takes place.

#### **5.10 Formal Complaint Investigation**

The investigation will be undertaken by an Investigating Officer appointed by Associate Directors of Nursing

#### **5.11 Letter of Response to Formal Complaint**

As soon as practical after the investigation is finished, the Investigating Officer will co-ordinate a written response, signed by the Chief Executive. If the Chief Executive Officer is unavailable, a Director acting on their behalf will be required to sign off the response.

The response will include:

- a reminder of the issues investigated, and the outcome sought.
- an explanation of how we investigated the complaint.
- the relevant evidence we considered.
- what the outcome is.
- an explanation of whether or not something went wrong that sets out what happened compared to what should have happened, with reference to relevant standards, policies and guidance.
- if something did go wrong, an explanation of the impact it had .
- an explanation of how that impact will be remedied for the individual.
- a meaningful apology for any failings.
- an explanation of any wider learning we have acted on/will act on to improve our service for other users .
- an explanation of how we will keep the person raising the complaint involved until all action has been carried out.
- confirmation that we have reached the end of our complaint procedure.
- details of how to contact the Parliamentary and Health Service Ombudsman if the individual is not satisfied with our final response.

## **5.12 Recommendations and Actions**

When the investigation is complete, the Investigating Officer will agree with the appropriate Service Lead the findings and where appropriate, agree achievable recommendations in the form of an action plan. This will identify the person to take the action forward and the timescales in which they need to be completed. These form part of the investigation report and are recorded within the document on the action plan.

The recommendations will be required to be SMART:

- Specific
- Measurable
- Achievable
- Realistic/ Relevant
- Timescale

Recommendations will be added to the Ulysses system, with regular reports sent to the directorates for them to monitor the progression of outstanding recommendations. All recommendations will be overseen by the Director of Operations monitored at the Directorate Safety and Quality Forums.

## **5.13 Extensions to Formal Complaint Timeframes**

Where circumstances are exceptional and require an extension, the Investigating Officer will be required to submit a written request to the Director of Operations for oversight and forwarded to the Patient Experience Team. The decision to agree an extension will sit with the Head of Patient and Carer Experience. It is the responsibility of the Investigating Officer to discuss and agree with the complainant the need for an extension, explaining the reasons for the delay and provide a new target timescale for completion.

It should be noted that extensions will not be agreed where there was delay in allocating an Investigating Officer. Extensions may be given for the following reasons:

- The complaint is potentially complex for example, involves a death, serious harm or involves several services.
- Where statements are required from key witnesses that are not available until a specified date. This should be identified at the start of the process and explained to the complainant.
- The Investigating Officer goes on sick leave after starting an investigation. In this case the Allocating Officer should consider reallocating the complaint and consult with the complainant.
- Where multiple agencies are involved, and timeframes are not the same.
- Where the Chief Executive Officer requests further clarification to finalise the response.

If the investigation and a final response cannot be completed within 6 months (unless we have agreed a longer timescale with the person

raising the complaint within the first 6 months) the Head of Patient Experience will write to the person to explain the reasons for the delay and the likely timescale for completion. They will then maintain oversight of the case until it is completed, and a final written response issued.

#### **5.14 Suspended Formal Complaints**

There may be an occasion when it is appropriate to suspend complaint investigations. Requests for suspension should in the first instance be sent to the PALS and Complaints Team. The decision to suspend will sit with the Head of Patient and Carer Patient Experience or by the Director of Nursing. The Patient Experience Team will suspend complaints automatically for the following reasons:

- The Investigating officer is unable to engage with the complainant, in this instance a decision will be made as to whether it would be appropriate to continue with the investigation.
- Complainant considered unwell and not be in the complainant's best interest to continue with the complaint.
- Awaiting consent for a third party to act on behalf of a patient.
- No contact from the complainant, in this instance a decision will be made as to whether it would be appropriate to continue with the investigation.
- Safeguarding issues.
- Complainant requests suspension.
- Police/criminal investigation.
- HR Investigation.

Suspended complaints are reviewed monthly if it is considered in the best interest of the patient, complainant or the Trust for the complaint to be closed, a written explanation will be provided to the complainant.

#### **5.15 Capacity, Consent and Confidentiality**

Where a concern or complaint is made on behalf of someone else, care must be taken not to disclose personal or health information to the complainant, unless the person has expressly consented to this disclosure. When appropriate, the complainant will be informed by the Patient Experience Team of the requirement to approach the person to seek their consent to share information in a full response to the complainant. Further guidance can be found in the Complaints Toolkit.

If there are concerns about whether the person about whom the concern/complaint relates has the capacity to give consent, then this matter should be discussed with the clinical team. A capacity assessment should be undertaken in relation to the information that is contained within the formal complaint and the requirements/scope of a complaint investigation (please refer to the Mental Capacity Act Policy). Further information is also available in the PALS and Complaints Toolkit.

### **5.16 Support for Staff Involved in a Complaint**

It is recognised that any incident, claim or complaint may be upsetting for staff and staff involved may need support. Complaints may also be stressful for the team as a whole, requiring openness and sensitivity and providing the opportunity to discuss the event. Support should be offered to any staff member involved immediately. If possible this should be through the line manager or the most senior person available. It is the duty of the manager of any staff member involved in an investigation to support those members of staff affected and to ensure that they are aware of all sources of immediate or ongoing support which they may access.

The Trust will offer support to staff, this support can include:

- Support from their manager
- Support at meetings through Trade Unions and/or Human Resources
- Occupational Health advice
- Confidential Counselling (COPE)

Staff may contact their professional registration bodies e.g. General Medical Council (GMC), Nursing Midwifery Council (NMC) for guidance and support.

### **5.17 Parliamentary and Health Service Ombudsman (PHSO)**

If, after all attempts at investigation and response through the formal complaints, the complainant remains dissatisfied with the response to their complaint; they have the right to ask the Parliamentary and Health Service Ombudsman to review the investigation into their complaint.

The complaint response letter will clearly inform the person raising the complaint that if they are not happy with the outcome of our investigation, they can take their complaint to the Parliamentary and Health Service Ombudsman

### **5.18 Multi-Agency Complaints and Complaints Relating to both the Trust and a Local Authority or other NHS Body**

The Trust is committed to joint working with partner agencies to resolve complaints, concerns and suggestions. In cases when the complaint relates in part to a Local Authority, or another NHS body, the involved Organisations must co-operate in coordinating the handling of the complaint and ensure that the complainant, with their agreement, receives a co-coordinated response to the complaint. This includes agreeing which body takes the lead in coordinating the handling of the complaint and communicating with the complainant.

Where the majority of issues within a complaint sits with one agency, that agency will lead on the complaint. If the Trust is requested to provide information for the complaint, the 35 day timescale will apply.

A discussion will take place on receipt of a multi-agency complaint. Staff will endeavour to work with private Organisations/agencies in the same way.

When the complaint relates entirely to services provided by another NHS body, or local authority, the Patient Experience Team will seek the consent of the complainant to forward the complaint to the relevant body within a target of 3 working days. The complainant's permission must be sought before forwarding the complaint to other Organisations for investigation.

#### **5.19 Handling Complex Persistent and Unreasonable Complaints**

In a minority of cases, complainants can become vexatious and persistent in pursuit of their complaint, despite reasonable attempts to resolve matters. This can result in a disproportionate amount of time and resources and may place undue strain upon the staff that are required to deal with them. Appendix B identifies complaints that may be considered unreasonable and suggests ways of responding to these.

#### **5.20 Duty of Candour**

Following the Francis Inquiry 2013, it is a requirement for Trusts to be open and transparent with people who use the services when a notifiable safety incident occurs. Duty of Candour is also part of NHS organisation's CQC registration requirement. Duty of Candour applies to incidents where moderate or severe harm or death has occurred as a result of a notifiable safety incident. Harm is defined as: All complaints that are connected to incidents or issues that result in moderate harm, severe harm or death to any patient, as defined in the Duty of Candour process, should be progressed in conjunction with the Being Open/Duty of Candour Policy (CWPT).

#### **5.21 Data Protection Act**

It is essential when dealing with complaints that staff comply with the Confidentiality Policy, Data Security and Protection Policy and Access to Health Records Policy. Any request to access clinical/medical records will be dealt with under the Data Protection Act (1998). As the Data Protection Act only relates to living persons, access to deceased patient's records is via the Access to Health Records Act (1990).

Any disclosure must be confined to that which is relevant to the investigation of the complaint and only disclosed to those people who have a demonstrable need to know it for the purpose of investigating the complaint. Records of complaints should not routinely be kept on patient's files.

#### **5.22 Complaints, Legal Action and Criminal Proceedings**

Where a complainant states they are commencing legal action against the Trust in relation to their complaint, the PALS and Complaints Team will notify the Deputy Chief Nurse and the Associate Director of

Corporate Affairs. Information collated during the investigation of a complaint may have to be disclosed when legal action is taken. It should be noted that the updated Complaints Regulations 2009 no longer state that the complaint should be halted where legal action is started.

It should also not necessarily be assumed that a complaint made via a Solicitor means that the complainant has decided to take legal action. If consent has been received, a response should be made in the normal manner.

If the subject of the complaint is a matter being referred to the police, the complaints procedure will be suspended pending the outcome of that investigation and the complainant will be informed of the reasons for this delay. Once the outcome is known, the complaint may continue if appropriate.

### **5.23 Discrimination**

Complainants need to feel confident that their care will not be adversely affected as a result of their having made a complaint. This commitment will be communicated to patients throughout the complaints process. The Investigating Officer will complete the Patient/Complainant Monitoring Information form, as part of the investigation process. Results will be monitored to identify any discrimination and action to be taken.

## **6 Consultation**

This policy has been developed in consultation with the Patient and Carer Experience Group, Managers, Clinicians and the Patient and Carer Assembly Steering Group. Following consultation, the policy was submitted to the Policy Ratification Group for approval/ratification.

## **7 Implementation**

This policy will be disseminated via the Trust's All User E-mail function and will be accessible to staff via the Trust's Intranet. It is the responsibility of managers to ensure that all staff are aware of Trust policies and of their individual responsibilities.

## **8 Training and Support**

Training relating to this policy will be provided through local Trust Induction and Investigating Officer Training.

## **9 Review**

This policy will be reviewed at least every three years or sooner where new or revised guidance/evidence requires changes to be made by the Chair of the Patient and Carer Experience Group.

## 10 Monitoring Compliance

| MONITORING COMPLIANCE WITH THE DOCUMENT                                                      |                                             |                                                        |                                  |                                                                     |                                                                                        |
|----------------------------------------------------------------------------------------------|---------------------------------------------|--------------------------------------------------------|----------------------------------|---------------------------------------------------------------------|----------------------------------------------------------------------------------------|
| Aspect of compliance or effectiveness being monitored                                        | Monitoring method                           | Individual / Department responsible for the monitoring | Frequency of monitoring activity | Group / Committee which will receive the findings monitoring report | Group / Committee / Individual responsible for ensuring that the actions are completed |
| Completion of investigations within the agreed time frame                                    | Monthly PT/EXP group Reports/ Annual Report | Patient Experience Team                                | Monthly<br>Annual                | Patient Experience Group                                            | Patient Experience Group                                                               |
|                                                                                              | Directorate Audits                          | Operational Directorates                               | Annual<br>6 monthly              | Safety and Quality Committee                                        | Safety and Quality Committee                                                           |
| Completion of the recommendations from complaint investigations within the agreed time frame | Monthly PT/EXP reports                      | Operational Directorates                               | Monthly                          | Patient Experience Group                                            | Patient Experience Group                                                               |
|                                                                                              | Annual Report                               | Patient Experience Team                                | Annual                           | Safety and Quality Committee                                        | Safety and Quality Committee                                                           |

## 11 References

The following documents and publications have been directly referenced or have been considered in the drafting of this policy:

- The Local Authority Social Services and National Health Service Complaints (England) 2009 regulations
- Clywd - Hart Report Review of the NHS Hospitals Complaints System (DH2013)
- Care Quality Commission Regulation 20 Duty of Candour Health and Social Care Act 2008 (Regulated Activities) Regulations 2014: Regulation 20
- NHS England Assurance of Good Complaints Handling for Acute and Community Care 2015
- The Principles of Good Complaint Handling (Parliamentary and Health Service Ombudsman, 2008)
- Duty of Candour NHS LA guidance
- Data Protection Act - In line with General Data Protection Regulations and current data protection legislation
- NHS Constitution (DH2012)

- NHS Litigation Authority Guidance about Complaints
- Being Open – Communicating Patient Safety Incidents with Patients and their Carers (NSPA 2009)
- Robert Francis Injury Report into Mid-Staffordshire NHS Foundation Trust (2010)
- Listening and Learning the Ombudsman Review of Complaint Handling by the NHS in England 2011-12
- HM Governments Prevent Strategy guidance for healthcare organisations (DH2011)

## 12 Trust Associated Documents

- Data Sharing Policy
- Being Open Policy (including Duty of Candour)
- Incident Reporting (SIRI) Policy
- Subject Access Request (SARs) Policy
- Care Records Management Policy
- Freedom to Speak Up Policy
- The Complaints and PALS Toolkit

## 13 Version Control

| Version | Date       | Author<br>(name and designation)                       | Status<br>(Draft/Approved) | Comments                                    |
|---------|------------|--------------------------------------------------------|----------------------------|---------------------------------------------|
| V7.0    | 04/12/2019 | Nicola Corbett<br>Head of Registration and Compliance. | Approved                   | PRG Chairs approval given.                  |
| V8.0    | 16/11/2022 | Judith Morris, Head of Patient and Carer Experience    | Draft                      | Policy review                               |
| V9.0    | 07/06/2023 | Judith Morris, Head of Patient and Carer Experience    | Approved                   | Policy Approved by PCEG and ratified by PRG |
| V9.1    | 14/11/2024 | Judith Morris, Head of Patient and Carer Experience    | Draft                      | Appendices A & B added.                     |
| V9.1    | 04/12/2024 | Judith Morris, Head of Patient and Carer Experience    | Approved                   | Approved by PCEG and ratified by PRG.       |

## 14 Equality Impact Assessment Form

|                                                                                                                                                                                                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
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| Name of new / revised Policy / Strategy / Procedure                                                                                                                                                                                                                                                                                                                                                                        |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Complaints policy                                                                                                                                                                                                                                                                                                                                                                                                          |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Name of Author                                                                                                                                                                                                                                                                                                                                                                                                             |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Judith Morris                                                                                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Directorate                                                                                                                                                                                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Nursing Safety and Quality Corporate                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| IDENTIFYING ALL THE AIMS OF THE POLICY / STRATEGY /PROCEDURE                                                                                                                                                                                                                                                                                                                                                               |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| <b>1.</b>                                                                                                                                                                                                                                                                                                                                                                                                                  | <b>CONSIDER THE EVIDENCE:</b><br><b>Does the Policy / Strategy / Procedure affect one group less or more favourably than another on the basis of the protected characteristics, Please explain?</b>                                                                                                                                                                                                                                                                                                                                                                                                                                      |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
|                                                                                                                                                                                                                                                                                                                                                                                                                            | <table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 40%;">Race</td> <td>No</td> </tr> <tr> <td>Religion or Belief</td> <td>No</td> </tr> <tr> <td>Gender reassignment</td> <td>No</td> </tr> <tr> <td>Sex</td> <td>No</td> </tr> <tr> <td>Sexual Orientation</td> <td>No</td> </tr> <tr> <td>Age</td> <td>No</td> </tr> <tr> <td>Disability<br/>(Learning disabilities, physical disability, sensory impairment and mental health problems)</td> <td>No</td> </tr> <tr> <td>Marriage and civil partnership</td> <td>No</td> </tr> <tr> <td>Pregnancy and maternity</td> <td>No</td> </tr> </table> | Race | No | Religion or Belief | No | Gender reassignment | No | Sex | No | Sexual Orientation | No | Age | No | Disability<br>(Learning disabilities, physical disability, sensory impairment and mental health problems) | No | Marriage and civil partnership | No | Pregnancy and maternity | No |
| Race                                                                                                                                                                                                                                                                                                                                                                                                                       | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Religion or Belief                                                                                                                                                                                                                                                                                                                                                                                                         | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Gender reassignment                                                                                                                                                                                                                                                                                                                                                                                                        | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Sex                                                                                                                                                                                                                                                                                                                                                                                                                        | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Sexual Orientation                                                                                                                                                                                                                                                                                                                                                                                                         | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Age                                                                                                                                                                                                                                                                                                                                                                                                                        | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Disability<br>(Learning disabilities, physical disability, sensory impairment and mental health problems)                                                                                                                                                                                                                                                                                                                  | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Marriage and civil partnership                                                                                                                                                                                                                                                                                                                                                                                             | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| Pregnancy and maternity                                                                                                                                                                                                                                                                                                                                                                                                    | No                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| 2. Where does responsibility for this Policy / Strategy / Procedure lie?<br><br>The responsibility for the policy review has been adopted by the ADO for RISE whoever this will be in conjunction with Adult Mental Health and implemented by senior leads and team leads within both directorates.                                                                                                                        |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| 3. How will the Policy / Strategy / Procedure be put into effect?<br><br>The process currently in place will be reviewed to ensure it meets the standard described in this policy.<br><br>All staff will be informed of the policy update through In-Brief all user email. Changes to the policy will also be discussed within the Patient and Carer Experience group                                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| 4. What criteria will you use to measure the effectiveness of the Policy / Strategy / Procedure?<br><br><ul style="list-style-type: none"> <li>Feedback from patients, carers and families who have raised a complaint will be used to inform the level of effectiveness of the policy.</li> <li>Feedback from staff will also be used to consider effectiveness and potential changes to the policy.</li> </ul>           |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |
| 5. Which other Trust's Policy / Strategy / Procedure does this link with?<br><br><ul style="list-style-type: none"> <li>Data Sharing Policy</li> <li>Being Open Policy (including Duty of Candour)</li> <li>Incident Reporting (SIRI) Policy</li> <li>Subject Access Request (SARs) Policy</li> <li>Care Records Management Policy</li> <li>Freedom to Speak Up Policy</li> <li>The Complaints and PALS Toolkit</li> </ul> |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |      |    |                    |    |                     |    |     |    |                    |    |     |    |                                                                                                           |    |                                |    |                         |    |

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|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p>6. How has there been involvement and/or consideration of stakeholders from different groups in the development of this Policy / Strategy / Procedure YES/NO.</p> <p>No.</p>                                                                                                                                                                                              |
| <p>7. How will information pre and post consultation be available e.g., on the Intranet?</p> <p>This has been discussed within Patient and Carer Experience Group and shared with service leads for consultation.</p> <p>All feedback received has been incorporated into the policy. If further feedback is received relevant changes to the policy will be considered.</p> |
| <b>IMPACT CHECKLIST</b>                                                                                                                                                                                                                                                                                                                                                      |
| <p>Are there factors other than elements of diversity which might help explain any adverse impact or use of this Policy / Strategy / Procedure e.g., the need to have a driving licence for a driving post (recruitment policy)</p> <p>N/A</p>                                                                                                                               |
| <p>How will the proposed Policy / Strategy / Procedure increase equality of opportunity for some groups or individuals? If so, explain how?</p> <p>It will seek to provide consistent approach to the handling of complaints.</p>                                                                                                                                            |
| <p>Are you confident that the Policy / Strategy / Procedure is lawful and can be justified?</p> <p>Explain</p> <p>Yes. This is in keeping with the NHS Complaint regulations.</p>                                                                                                                                                                                            |
| <b>ALTERNATIVES CHECKLIST</b>                                                                                                                                                                                                                                                                                                                                                |
| <p>Are there any alterations or additional measures which would remove or reduce adverse impact without effecting the Policy / Strategy / Procedure overall aims?</p> <p>To support people who complain if their first language is not English, arrangements would be made to book an Interpreter.</p>                                                                       |
| <p>If this Policy / Strategy / Procedure positively impacts a particular group, will any benefit to them outweigh the costs other groups? please explain.</p> <p>N/A</p>                                                                                                                                                                                                     |
| <p>If you do not make above alterations, what are the consequences for the Partnership Trust or those individuals/groups? e.g., employment tribunal</p> <p>N/A</p>                                                                                                                                                                                                           |
| <b>DECIDE WHETHER TO ADOPT THE POLICY / STRATEGY / PROCEDURE</b>                                                                                                                                                                                                                                                                                                             |
| <p>What are the main findings of the consultation?</p> <p>N/A</p>                                                                                                                                                                                                                                                                                                            |

|                                                                                                                              |
|------------------------------------------------------------------------------------------------------------------------------|
| How will you change the policy / strategy / procedure to consider feedback from consultation and other information gathered? |
| Further changes to the policy would be considered from any feedback received by complainants and staff.                      |
| Who will be involved in the decision-making process of whether to adopt the Policy / Strategy / Procedure and why?           |
| N/A.                                                                                                                         |
| <b>MONITORING</b>                                                                                                            |
| Consider piloting the Policy / Strategy / Procedure to assess impact why Yes or No                                           |
| N/A                                                                                                                          |
| What criteria will be used to assess the effects on equality of opportunity and who will carry out the assessment?           |
| Rano Bains                                                                                                                   |
| How often will the Policy / Strategy / Procedure be reviewed and by whom?                                                    |
| Every 3 years by the policy author.                                                                                          |

|                                                                                    |                                                                                                                                                                               |
|------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Date of Assessment: (dd/mm/yyyy)                                                   | 05/05/2023                                                                                                                                                                    |
| Names and designations of the 3 people who contributed to this full EIA assessment | 1. Judith Morris, Head of Patient and Carer Experience<br>2. Lisa Taylor, PALs and Complaints Coordinator<br>3. Janet Kenworthy Harvey, Lead Nurse, Mental Health Directorate |
| Head of Equality, Diversity and Inclusion                                          | Rano Bains                                                                                                                                                                    |
| Date of Ratification:                                                              | 07/06/2023 (04/12/2024 minor amendment)                                                                                                                                       |
| Date of first review:                                                              | 07/06/2026                                                                                                                                                                    |

If you have identified a potential discriminatory impact on this procedural document, please refer it to the author of the policy or strategy, together with any suggestions as to the action required to avoid / reduce this impact. For advice in respect of answering the above questions, please refer to the guidance notes. **If the document affects one group less or more favourably, you MUST complete the full EIA form (i.e., if you have answered 'Yes' to any of the above).** The full EIA form can be obtained from the Equality, Diversity and Inclusion Department/website.

This policy, strategy, procedure or function has to go to the Head of Equality, Diversity and Inclusion for final sign off.

Please return a copy to [rano.bains@covwarkpt.nhs.uk](mailto:rano.bains@covwarkpt.nhs.uk)

## Appendix A

### Duties and Responsibilities of Operational Staff / Team

|                                           |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
|-------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p><b>The Patient Experience Team</b></p> | <p>The Patient Experience team will provide advice and support to staff when dealing with an informal concern and formal complaints.</p> <p>The Patient Experience Team will assess and consider all aspects of a complaint situation for example, a safeguarding, legal, or Serious Incident Requiring Investigation (SIRI). These complaints will be placed on hold until the relevant team confirms it is appropriate to proceed with the complaints process. All concerns or complaints received into the PALS office via email, post or telephone will be acknowledged by the team and will ensure comments; concerns or formal complaints are recorded.</p> <p>The team is responsible for creating and maintaining an electronic database (Ulysses) and folders to contain all relevant information relating to a complaint or concern.</p> <p>They will send the details of the concern to the relevant team / Director as per the distribution list.</p> <p>For formal complaints the Patient Experience team will meet with the Investigating Officer (IO) to scope the complaint to identify the elements of complaint to be investigated and will meet with the IO on a regular basis throughout the investigation to offer advice and support.</p> <p>For formal complaints the Patient Experience team will be responsible for forwarding the final draft response letter to the CEO for sign off.</p> |
|-------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

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|-----------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Associate Directors of Nursing / Directors of Operations</b> | <p>Associate Directors of Nursing / Directors of Operations have overall responsibility for the implementation of this policy within their Directorate. This involves ensuring that all staff are fully conversant and compliant with the policy.</p> <p>The Associate Directors of Nursing / Directors of Operations or nominated deputy are responsible for:</p> <ul style="list-style-type: none"> <li>• To ensure information about how to make a complaint is available to complainants by making sure those posters and leaflets are available in their area of responsibility.</li> <li>• To ensure that their staff have the necessary skills and knowledge to deal with concerns and complaints and know how to access support from more senior staff.</li> <li>• Identifying an appropriate Investigating Officer to investigate formal complaints / CQCs / MP enquiries / complaint reviews.</li> <li>• To continue to liaise with the Investigating Officer during the investigation and offer support as needed.</li> <li>• Agree any recommendations identified following the investigation and agree the action plan.</li> <li>• Agree the draft final response letter and forward to the Patient Experience team.</li> <li>• Ensuring that reports, findings, and recommendations are communicated in their Directorates to facilitate Organisational learning.</li> <li>• Ensuring the implementation of any service improvements as a result of recommendations are completed within the identified timescales.</li> <li>• To ensure evidence of actions have been completed are forwarded to the Patient Experience team.</li> <li>• To ensure that the Action Tracker is monitored within their Directorate and complaint issues and themes are discussed in Safety and Quality Forums.</li> </ul> |
| <b>Investigating Officers</b>                                   | <p>The role of the Investigating Officer is to undertake an investigation into the issues raised by the complainant.</p> <p>They are responsible:</p> <ul style="list-style-type: none"> <li>• Making contact with the complainant to confirm the elements of complaint. This must be included in the complaints file.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |

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|--|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|  | <ul style="list-style-type: none"> <li>• The Investigating Officer will verify consent with the patient.</li> <li>• The Investigating Officer is responsible for ensuring that the complainant is kept up to date with progress at intervals agreed with them at the start of the complaint process, ensuring a thorough, fair, factual and timely investigation is carried out into the complaint.</li> <li>• The Investigating Officer will complete the investigation and draft response within 27 working days using the Trust template and agree any recommendations with the Associate Director of Nursing / Directors of Operations or their identified deputy for the Directorate concerned.</li> <li>• The Investigating Officer will ensure that patients, carers, and any relevant staff are kept fully informed of the complaint's process, taking into consideration the assistance required to support those complainants who may need communication support.</li> <li>• The Investigating officer will inform the complainant of any potential delays in the investigation at the earliest opportunity and renegotiate the timescale for response.</li> <li>• Where the complaint involves clinical issues, the findings and the response must be shared with the relevant clinicians to ensure factual accuracy. The ADN will ensure that anyone responsible for specific actions is made aware of them.</li> <li>• The Investigating Officer must provide the Patient Experience Team with copies of all documents used in the investigation, e.g. copies of all clinical records accessed, witness statements, details of whether the elements are upheld or not upheld and the action plan.</li> </ul> |
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## Appendix B

### Handling Complex, Persistent and Unreasonable Complaints

In a minority of cases, some people can become unreasonably persistent in pursuit of their complaint despite reasonable attempts to resolve matters. This can result in a disproportionate amount of time and resources and may place undue strain upon staff who are required to deal with them. The aim of this guidance is to identify situations where complainants may be considered persistent or unreasonable and to suggest ways of responding to these. It is important to remember that a person making a complaint may be distressed due to events that have happened, they may be bereaved, or have health problems and, therefore, they may act out of character at times of stress. Some may find it difficult to communicate; others may appear to show aggression, have a mental illness or be lonely and lack support.

These factors should be taken into consideration when dealing with any member of the public who makes a complaint and before implementing this policy. It should be emphasised that this process should only be used when all reasonable measures have been taken to try and resolve complaints through the NHS Complaints Procedure. The Director of Nursing will only implement this process following discussion with the Chief Nurse.

Definition of a persistent or unreasonable complainant – Complainants may be considered persistent or unreasonable where previous or current contact with them meet two or more of the following criteria (or one, in extreme circumstances): Where complainants.

- Persist in pursuing a complaint where the complaints procedure has been fully and properly implemented and exhausted or refuse to accept the remit of the complaints procedure.
- Change the substance of their complaint or add trivial or irrelevant issues to prolong contact with the Trust. Care must be taken not to disregard new issues where they are genuinely identified late in the investigation and are significantly different to the original complaint. These will need to be addressed as separate complaints.
- Repeated focus on conspiracy theories and/or will not accept documented evidence of treatment given e.g. information in a patient's records, computer records or do not accept that facts can sometimes be difficult to verify when a long period of time has elapsed.
- In the course of pursuing a complaint, has made an excessive number of contacts (or made multiple complaints) with the Trust, placing unreasonable demands on staff. Such contacts may be in person, by telephone, letter, or electronically. Discretion must be exercised in deciding how many contacts are required to qualify as excessive, using judgement based on each case.
- Do not clearly identify the precise issues, which they wish to be investigated despite reasonable efforts made by Trust staff or advocacy service to help them.
- Have harassed or been verbally abusive or aggressive on more than one occasion towards staff dealing with their complaint, or failed to engage with staff

in a manner which is considered appropriate e.g. repeated use of unacceptable language. Staff must recognise that complainants may sometimes act out of character at times of stress, anxiety or distress. All incidents should be reported via Ulysses.

- Focus on a relatively trivial matter to an extent that is out of proportion to its significance. It is recognised that this can be subjective and, therefore, careful judgment must be used in applying this criterion.
- Have threatened or used actual physical violence towards staff at any time. If this applies, no other criterion is necessary to invoke the policy. All incidents should be documented.
- Known to have recorded meetings, face-to-face or telephone conversations without the prior knowledge and consent of the other parties involved.
- Display unreasonable demands/expectations and fail to accept that these may be unreasonable (e.g. insist on responses to complaints or enquiries being provided more urgently than is reasonable or usual recognised practice).

Where complainants are identified as persistent or unreasonable, in accordance with the above criteria, the Director of Nursing will discuss, and agree any further action with the Chief Nursing Officer. The following will be considered prior to taking action:

- Ensure that the complainant's issue have been dealt with appropriately and that reasonable action has followed;
- Ensure that any new or significant issues have been considered;
- Apply criteria with fairness, and due consideration for the complainant's circumstances (any known physical or mental health conditions, impact of bereavement or sudden change in lifestyle that may explain their behaviour;
- Consider the proportionality of any restrictions enforced in comparison with the level of unreasonableness or behaviour of the complainant; Consider whether there are further actions that can be taken to resolve the matter; and
- Try to resolve matters, before invoking the policy, by outlining to the complainant the type of behaviour expected if the Trust is to continue investigating the complaint under the NHS Complaints Procedure.

Where all the above have been considered and the complaint is considered persistent and/or unreasonable, the Chief Executive or Chief Nurse will notify the complainant in writing of any action taken and the reasons why the Trust feels that this is necessary. Details of this will be recorded in the complaints file and complaints database.

The options to consider are as follows:

If a complainant is abusive or threatening, it is reasonable to require him/her to communicate only in a particular way. The Trust may, therefore, withdraw contact with the complainant in person, by telephone, by fax, by letter or any combination of these provided that one form of contact is maintained. Please refer to the Trusts Respecting our people addressing unacceptable behaviours framework for further guidance. It may also be helpful to nominate one individual to maintain contact; this will normally be the Head of Patient & Carer Experience

- Place time limits on any telephone conversations and personal contact and only in the presence of a witness.

- Require any future contact via an advocate e.g. Community Health Council, Healthwatch Shropshire, etc.
- Advise the complainant that the Trust will not deal with correspondence that is abusive or contain allegations that lack substantive evidence.
- Notify the complainant in writing that the Chief Executive has responded fully to all the issues raised and that continuing contact on the matter will serve no useful purpose. The complainant will be notified that the investigation into their complaint is complete and that any further correspondence relating to the issues previously addressed will be acknowledged but not answered.
- Temporarily suspend all contact with the complainant or investigation of the complaint whilst seeking legal advice.
- In extreme circumstances inform the complainant that the Trust reserves the right to pass persistent or unreasonable complaints to the Trust's solicitor and this may result in legal action.
- Where staff have been threatened, assaulted or harassed the Trust will consider whether it is appropriate to refer the matter to the police for investigation.
- HR to be included in discussions if deemed appropriate.

Having deemed a complainant as persistent or unreasonable, this status may be withdrawn at any time. This should be exercised with discretion where for example, the complainant agrees and demonstrates a more reasonable approach or they submit a further complaint for which the normal complaints procedure would appear appropriate. The Head of Patient & Carer Experience will discuss options with the Director of Nursing and if considered appropriate, the Trust's complaints procedure will apply and the complainant notified.

### **Complex Complaints**

Where the complaint is particularly complex or where serious patient safety issues have been identified the appropriate directorate should complete an implementation SPOC / consideration of complex complaint Escalation Form signed by the Associate Director and forward it to the PALS & Complaints team. The Director of Nursing, the Head of Patient & Carer Experience, the Head of Risk Management, the relevant Directorate Lead (this could be the Director or an Associate Director), a member of the patients clinical team and the Investigating Officer will meet to discuss options, including the appointment of a Single Point of Contact (SPoC) to oversee the complaint investigation and be the SPoC for the complainant. The complainant will be advised that any future communication with the Trust should only be through the SPoC who will triage and signpost accordingly.

### **Support for staff**

The Trust recognises that persistent or unreasonable complainants can place undue stress upon staff who may require additional support from their immediate manager.

The Trust recognises it can be extremely distressing for staff when they are involved in a complaint investigation whether this is an internal review, or an external investigation conducted by the Parliamentary & Health Service Ombudsman.

Therefore, staff involved in any part of the complaints process may require additional support from their immediate line manager throughout this process.

### **Immediate Support**

The line manager should be the first point of contact for staff involved in a complaint and will provide appropriate support to any member of staff throughout the life of the complaint.

### **Further action and support where staff are experiencing difficulties**

Managers and Heads of service have a responsibility to ensure that their staff are appropriately supported during this process and in serious cases, or where the member of staff is experiencing difficulty, must advise staff of the availability of the support available. On-going support is available from the following:

- Occupational Health Service
- The Chaplaincy Team, who are available for all staff to speak on an informal and strictly confidential basis.
- Confidential Counselling (COPE)

Some staff may still find it difficult to work due to stress even when support, information and guidance are offered. Other alternatives should be discussed with the staff member, and this may result in a risk assessment being undertaken by their line manager supported by Human Resources.

### **Roles and Responsibilities of the Patient Experience Team**

- Arrange the initial Complex Case Review meeting.
- To be the scribe during the Complex Case meeting and arrange any further meetings.
- An action plan will be developed and be monitored by the Patient Experience Team.
- Will request regular updates from the Investigating Officer and the Director of Operations.
- To complete an update log of all correspondence / communication received from and sent to complainant.
- To log any new concerns identified in correspondence received from the complainant.

## Request for implementation of Vexatious complaint /complex case

Date:

|                                                                                                                |  |
|----------------------------------------------------------------------------------------------------------------|--|
| Name and Job title of referrer.                                                                                |  |
| Email address                                                                                                  |  |
| Directorate & Service                                                                                          |  |
| Details of previous PALS & Complaints (previous six months ) include ref number , date and summary of concerns |  |
| Details of events leading up to the request (include timeline)                                                 |  |
| Reasons why the process is being considered or withdrawn.                                                      |  |
| Details of staff who have been involved in case to date.                                                       |  |
| Complainant details ( Name, address )                                                                          |  |
| Patient details (if different from complainant) Name , address, date of birth                                  |  |
| ADO/ADN Signature                                                                                              |  |
| Additional information                                                                                         |  |

To send the completed form to [PALS.Complaints@covwarkpt.nhs.uk](mailto:PALS.Complaints@covwarkpt.nhs.uk)

**To be completed by Head of Patient & Carer Experience**

**Date:**

|                                                                                                                                                                               |                |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|
| <b>Risk assessment completed</b>                                                                                                                                              | <b>Outcome</b> |
| <b>GREEN</b> = Advice given<br><b>AMBER</b> = seek advice from DoN<br><b>RED</b> escalate DoN arrange meeting to discuss case.                                                |                |
| <b>Meeting arranged</b><br><b>ADN/ Referrer /DON/ H of PE, H of Risk management</b><br><b>(additional safeguarding, psychology, solicitor, Comms )</b><br><b>as required.</b> |                |
| <b>Outcome of meeting</b>                                                                                                                                                     |                |
| <b>Action to be taken.</b>                                                                                                                                                    |                |
| <b>Details of SPoC):</b>                                                                                                                                                      |                |
| <b>Details of the proposed future communication plan (include frequency and method of contact</b>                                                                             |                |
| <b>Letter issued.</b>                                                                                                                                                         |                |
| <b>Details of actions to be taken if the individual fails to comply.</b>                                                                                                      |                |
| <b>Considered and agreed by</b><br><b>Director of Nursing</b><br><b>Signature and date</b>                                                                                    |                |